

Understanding **BPD**

When Your Parent Has BPD

What is happening at home, why you didn't cause it, and what you can do starting this week.



Questions you have probably asked

Why is Mom fine one minute and furious the next?

Did I do something to cause this?

Is it always going to be like this?

If your mom or dad has borderline personality disorder, or you think they might, you have likely asked all three. Most writing about BPD is for the person who has it, or for a husband or wife. Very little is written for the teenager down the hall. This guide is for you.

It covers what BPD is, why your parent can seem like two different people, and the things research says help kids in your position. Every research claim is cited at the back. The example conversations were written for this guide, built from those findings, and they are a starting point you can put in your own words.

If someone is in danger right now

Call or text 988, any hour of the day. If someone could die, call 911. More numbers are near the end of this guide.

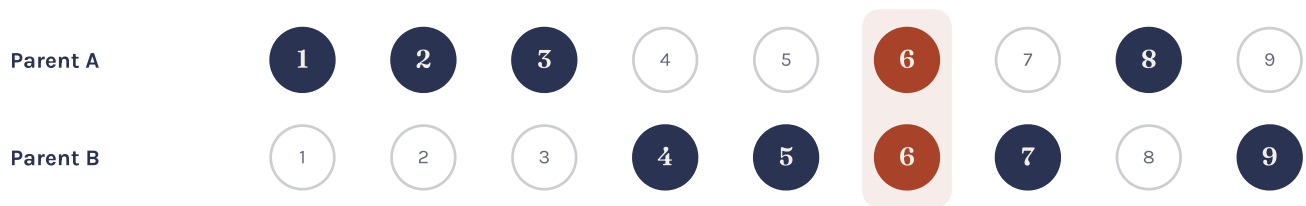
What is BPD, in plain words?

Borderline personality disorder is a mental illness that affects how a person handles emotions. Feelings arrive faster, hit harder and take longer to fade than they do for most people. Relationships feel high-stakes, because the person is often terrified of being left.

Doctors diagnose it from a list of nine traits. A person needs five or more. Here is the list in everyday words.

- 1 Working hard to avoid being left, even when nobody is leaving.
- 2 Relationships that swing between adoring someone and resenting them.
- 3 A shaky sense of who they are.
- 4 Impulsive choices that can hurt them, like overspending or reckless driving.
- 5 Self-harm, or threats or attempts of suicide.
- 6 Moods that change fast, often within hours.
- 7 A long-lasting feeling of emptiness.
- 8 Intense anger that is hard to control.
- 9 Under heavy stress, feeling paranoid or unreal.

Same diagnosis, one trait in common



Each circle is one trait from the list above. Both parents qualify for the diagnosis, and the only trait they share is number six. Five or more out of nine can be met in 256 different ways.

So your parent has their own version of BPD. You understand it by learning their patterns: what sets them off, how long it lasts, and what helps them come back down.

Why does my parent seem like two different people?

Researchers who study mothers with BPD describe a pattern. The same parent can swing between two modes (Stepp and colleagues, 2012). In one, they are controlling and harsh: criticizing, checking up on you, angry about small things. In the other, they pull away. They are distant, shut in their room, hard to reach.

A lot of kids decide one of these is the real parent and the other is an act. Both are real. Both come from the same illness, and neither one is a verdict on you.

BPD also pushes people to see others in all-or-nothing terms. On Monday you are the only one who understands them. On Tuesday you are selfish and ungrateful. You stayed the same person between Monday and Tuesday. Their view of you swung, and it will likely swing back.

Did I cause this?

No. A saying from Al-Anon, a support group for families, fits here: you didn't cause it, you can't control it, and you can't cure it.

BPD grows out of a mix of genes and life experience, usually long before a person has children. Nothing you said at dinner caused it, and no amount of perfect behavior will cure it.

How understanding the illness helps you

Of everything in this guide, this has the most research behind it. A review of 37 studies found that kids whose parent has a mental illness do better when someone explains the illness in words they can follow (Van Schoors and colleagues, 2023). In programs that taught kids about a parent's illness, kids were about a third as likely to develop depression or anxiety over the next year or so (Havinga and

colleagues, 2021). Those programs were for parents with depression or anxiety. Research on BPD families is newer, and understanding is still the best place to start.



Here is what changes once you understand it.

- You stop taking the blame. A blowup becomes a symptom you can name.
- Your parent becomes more predictable. You learn the triggers and roughly how long a bad stretch lasts.
- You have words for it. “My mom has an illness called BPD” is a sentence you can say to a counselor or a friend.
- You can see the person underneath. Your parent is more than their worst days.

What tends to set things off?

The biggest trigger is fear of being left. People with BPD watch closely for signs that someone is pulling away, and they react hard when they think they see one.

That puts teenagers in a tough spot. Growing up means pulling away a little. You close your door, you want time with friends, you keep some things private. All of that is normal. To a parent with BPD, each one can feel like the first step of being abandoned. No study has tested this exact point. It follows from how the illness works, and it explains a lot of fights that seem to come from nowhere.

Knowing this won't stop the reaction. You can see it coming, though, and you can choose how you respond.

You are heading out with friends and your parent is upset

Instead of

“Why do you always do this? I'm allowed to have a life.”

Try

“I'm going to Maya's until nine. I'll text you from there, and I'll be home at nine.”

A set time and a promise to check in speak to the fear underneath. An argument about your rights skips right past it.

Your parent says you don't love them

Instead of

“That's ridiculous. You're being crazy.”

Try

“I do love you. I can tell you're really scared right now.”

While the feeling is this strong, arguing the facts goes nowhere. Naming the feeling often lowers the temperature.

Your parent goes silent after an argument

Instead of

Following them from room to room until they talk.

Try

“I’m going to do my homework. I’m here when you want to talk.”

You can't pull someone out of a shutdown. Telling them where you are, and that you are staying, is enough.

What can I actually do?

More than you might think. Research on kids whose parent has a mental illness keeps finding the same few things that protect them, and you can start most of them this week.

Pick one steady adult

It could be your other parent, a grandparent, an aunt or uncle, a coach, a teacher or your school counselor. Who it is matters less than how often you see them. Support that shows up regularly makes the biggest difference (Van Schoors and colleagues, 2023).

Who is in your corner?



● People I see every week

● People I could call

● Professionals

School counselor, a doctor, 988

Fill this in with a pencil. One name in the inner ring is a strong start.

Keep your own life going



When researchers asked kids in your position what helped, they named time with friends and ordinary activities (Bee and colleagues, 2014). Kids who coped by accepting what they couldn't change, looking for the good in things and taking their mind off it had less anxiety and depression (Van Schoors and colleagues, 2023). Practice counts as coping. So does a Friday night with friends. You are allowed to have a good time while things at home are hard.

Tell one safe person

Kids with a parent who has a mental illness often keep it secret, because they worry what people will think (Dobener and colleagues, 2022). Carrying that secret alone wears you down. You don't need to tell everyone. One safe person is enough: a friend you trust, a relative, a counselor.



A school counselor is a good first choice. Talking with students about hard things at home is part of the job.

Have I become the parent?

In a review of 14 studies, young people with a parent who had a mental illness described taking over their parent's job so often that the researchers added a new category to the standard model of how kids cope (Sawitzki and colleagues, 2025). The name for it is role reversal.

It looks like calming your parent down every night, keeping the peace between your parents, running the house, or being the person your parent tells everything. Kids who do this are often praised for being mature. It is still too much weight for someone your age.

Whose job is it?

An adult's job	Your job
Calming your parent down every night	School and homework
Keeping the peace between your parents	Friends, practice, the things you love
Running the house	Sleep
Being the one your parent tells everything	Telling an adult when things are bad
Deciding whether your parent needs treatment	Asking for help for yourself

If the left column sounds like your week, tell your steady adult. Handing one of those jobs back to a grown-up is a real change.

Should I try to talk my parent into therapy?

You can, and you can also leave it to an adult. Your parent's treatment belongs to your parent and their doctor.

If you do raise it, research on adults offers a few clues. People who were listened to without judgment and gently pointed toward help were much more likely to see a doctor or therapist afterward (Morgan and colleagues, 2025). Most people who skip treatment believe they don't need it, or want to handle it alone (Mojtabai and colleagues, 2011), so one conversation rarely settles it. In one study, mothers with BPD said they wanted help being better parents (Zalewski and colleagues, 2015). That is often a better way in than the word “disorder.”

Bringing it up for the first time

Instead of

“You need help. I looked it up, and I think you have BPD.”

Try

“I love you, and I can see how hard things have been. Would you talk to someone? I'd feel better knowing you had support.”

A diagnosis from your own kid lands as an accusation. Worry from your own kid is much harder to argue with.

In the middle of a fight

Instead of

“If you don't go to therapy, you're going to lose us.”

Try

Saying nothing about therapy until you are both calm.

About one in five parents in one study of public mental health care feared losing custody of their kids over treatment (Busch and Redlich, 2007). A threat feeds that fear and makes treatment less likely.

A trusted adult can carry this conversation better than you can. In research trials, a short family session where a counselor explains the illness to everyone helped kids feel less anxious (Solantaus and colleagues, 2010). Ask your steady adult whether your parent's therapist, or a counselor, could set one up.

When home doesn't feel safe

Understanding the illness helps you make sense of your parent's behavior. Being hurt is still never okay. If a parent hits you, threatens you, or leaves you without food or a safe place to sleep, tell an adult you trust or call one of these numbers.

Free, confidential, and open all day and night

- 988 Suicide & Crisis Lifeline: call or text 988. You can call about your parent or about yourself.
- Childhelp National Child Abuse Hotline: call or text 1-800-422-4453.
- Crisis Text Line: text HOME to 741741.
- If anyone is in immediate danger, call 911.

Will my parent ever recover?



There is real reason for hope.

A review of 19 long-term studies found that remission from BPD, meaning a person no longer meets the criteria, is common. Once people reach remission, few slip back (Ng and colleagues, 2016). Treatments such as dialectical behavior therapy, or DBT, were built specifically for BPD.

Recovery in everyday life, like steady relationships and work, can take longer than the symptoms do. Your parent may have hard seasons ahead. The illness can still change, and your family can change with it.

Your own future belongs to you as well. Understanding the illness, a steady adult, friends, and your own ways of coping are the same things researchers link to kids doing well when a parent is unwell (Van Schoors and colleagues, 2023). Every one of them is already in your hands.

Where to start this week

Five small steps. None of them depend on your parent changing.

- ☐ Write one name in the inner ring of your corner.
- ☐ Tell that person one true thing about what happens at home.
- ☐ Make one plan with friends, and keep it.
- ☐ Pick one job from the adult column and hand it back to a grown-up.
- ☐ Save 988 and 1-800-422-4453 in your phone.





Meet Marley

This project started with one family trying to make sense of a diagnosis.

Marley grew up inside a family where emotions ran high and nobody had a word for why. That question led her to spend a summer buried in clinical research on borderline personality disorder, and to build this series of guides for other families doing the same thing hers did.

Each guide is written from the peer-reviewed literature rather than from anecdote, and every claim is traceable to the sources listed at the back. They are free, and they are meant to be shared with the people who need them.

“Nobody in my family had the language for what was happening. These guides are the thing I wish someone had handed us.”

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Most of the research on programs for children studied parents with depression or anxiety; studies of children of parents with BPD are fewer and more recent. The example conversations and the three diagrams were written for this guide. They follow the research above, and they were not themselves tested in a study. Crisis numbers were checked as current at the time of writing.